

Tue, Feb 07, 2023 11:05AM • 40:51

Interviewer 00:00

very quickly. There you go. Okay. And to start with again, I just want to get a better sense of your practice. You are currently primarily providing EFT is that correct?

Therapist 00:16

Yes, that's correct

Interviewer 0:17

All right. And you are currently based here in [Country A], but is your type of clients that we provide your services to? Mostly [NATIONALITY] as well? Or?

Therapist 00:32

No, they are, Well, they are expats most of the time. But yeah, most most of the times, not always one of the partners is [Nationality A]. In my bet so one is from some from outside of [Country A], and one partner is [Nationality A] most of the times but I also have couples from the that are both not from [Country A]. Or they both are [Nationality A]. That's more unusual, but I do have. So it's mixte.

Interviewer 01:13

And do you do you still provide a couple therapy to [Nationality B]? Or is that usually not your client?

Therapist 01:22

Not might Yeah, most of my clients are, I think are expats. And in this for couples, specifically, I would say that it's more there's more one, one is [Nationality A] and the other one is from somewhere else. But no, not so many [Nationality B]. I don't have so many [Nationality B] now.

Interviewer 01:47

So I assume you started your clinical practice here in [Country A], and not when you were still in [Country B] .

Therapist 01:57

Sorry?

Interviewer 01:58

You started your clinical practice well, when you were already here in [Country A], and not when you were in [Country B]?

Therapist 02:02

No, I've started in [Country B]. And I Yeah, so I've started my clinical practice in [Country B]. And I also worked in the [Country C] for [1-5] years. Yeah. [1-5] years. Yeah. And then I've started then had also my practice back in [Country B], and then yeah, so yeah, I worked in three different places.

Interviewer 02:34

All right. So when the clients come to you, what is usually the presenting issue or the reason that they seek your service? And I know there's going to be a broad range, but are there typical cases of why they come to you?

Therapist 02:52

The way they they well they the typical is that they are having the if they present as difficulty with their communication, I think most of the times but yeah, they are have they are experiencing stress in the way they interact with each other. I would frame it like that.

Interviewer 03:21

Yeah. All right. And do you do a formal assessment ?

Therapist 03:31

A formal assessment with couples No.

Interviewer 03:34

So, how would you how do you do the assessment whether formally or informally,

Therapist 03:41

I assess the couple first, well informally, I tried to understand I asked them, how they interact asking like examples from their interactions. And I, yeah, I basically asked for examples of their interactions of what they do. So I go, I understand I go specifically to how they behave, how they feel, what what their thoughts are, in the in these moments of when they say this is a moment of stress in our interaction. So I go to these specific points with both of them and I also assess them individually in individual sessions. And then what I do is mostly in relation to their attachment history. I assess their experience with relationships with significant relationships as they were growing up and in also in adulthood, so with their parents or caregivers, but also all their romantic relationships.

Interviewer 05:21

All right. In the course of your practice, have you used relationship skills?

Therapist 05:34

Relationship skills? I used of Sue Johnson Maybe very few times are not that much. But I used one in relation to understand if they feel their partner is responsive and assessable. It's I forgot the name, but it's not a formal skill. It's something she has in her book. So I use that. But yeah, but I don't think I used others. No.

Interviewer 06:20

Okay. And how was your experience with that scale then?

Therapist 06:29

It was a way to maybe open conversations with and to understand more from clients that maybe were difficult for me to, to assess in a different way, but I didn't find it helpful. That's maybe why I don't use it that often.

Interviewer 06:54

Can you can you expound more on why you didn't find it helpful?

Therapist 07:02

I think it was called to, for clients to say, what I don't remember how was this? The scale was not? Was not? How do you call Likert Likert scale? Like it wasn't? They had only the option? Yes or no? Like, do you feel your partner is that? And clients found that it was really it was only a way of opening conversations? Because they said, well, it's not yes or no. It's in these moments. So I found that in the end, I didn't need that to do what we were doing, because so I thought it for them wasn't helpful. There to frame it as yes or no. And for me, yeah. It was like what I was doing, I could do in a different way.

Interviewer 08:00

And would you remember how long the scale was?

Therapist 08:07

There were three categories: accessibility responsiveness. And I don't remember the other one in engagement. I don't remember. I don't know, maybe I don't remember exactly. But I think Max 20 questions in total, something like that.

Interviewer 08:27

All right. And this, you didn't pay for the scale. Right? You didn't buy it then?

Therapist 08:33

No, it was something it was in a book that sometimes I give clients to read. And they they did the exercise, like as part of this reading the book. And so we did it online, and I was just going through the questions with them to see how they responded. So

Interviewer 08:54

Interesting. Okay. And I wanted to ask as well across the years and I think you've seen this in my email as well over the years different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. But I wanted to get your view about it. For you, what is relationship quality?

Therapist 09:23

Yeah, that's a big question but it's the main thing is when partners care about the other like the how the other the pain basically how if they look and if they have that sense that you're, what what you feel with your hurt matters to be like when they, when they have that, I think. There is a potential, like, especially because I deal with clients that are not satisfied with their relationship, I think that shows that there there is a, there is a quality there that there is potential for them to work on their problems. That's the basic thing. But for relationship quality, yeah, I think it's this attunement too. Because partners can always hurt themselves. And there is nothing that we can do to prevent that from happening. But it's when these matters, and they are, they are attentive to that, that something happens. And that's take care of that. And you felt like that they they are basically attentive and responsive to that, that they see

their partner in pain and and they turn to them they don't avoid or they don't diminish that something that they shouldn't be feeling or something that doesn't make any sense. And this is the I think, what shows that there is quality?

Interviewer 11:28

And would you say that is the most important thing in our relationship being attentive, or making, you know, like, the partner knowing that the hurt matters to them? Would you say that is the most important characteristic in a relationship?

Therapist 11:51

I think so. I feel that this I mean, it's there are many things that are of course, important in relationships, so many things. But I think that in the end, if you have to see what what is what is really important for people to, to live a meaningful life and have a meaningful life together is when they are able to do that to see what oh, we this matters to you. This hurts you this is not okay. When they are able to do that. That's yeah, I think that's the most important thing.

Interviewer 12:33

You said there, of course, many other important things. What would these be for you as well? Like, what does matter in a relationship for you?

Therapist 12:49

Is what matters is feeling? Well, it's important. You mean you mean in terms of feelings? I don't know. It's what what is important, it's feelings, that you are seeing that you are important that you are valued, that you are that you are love that you matter that the things you Yeah, I think these are the feelings that are important to have, that you are seeing that you are that you matter that you are important valued.

Interviewer 13:47

Yeah. And some really some that I've talked to as well make the distinction between behaviors that make the relationship survive, and behaviors that make the relationship flourish. For you, do you? Is there that sort of distinction as well? Are there behaviors that you know, that would make a relationship survive that doesn't necessarily make it flourish or that makes it flourish but doesn't necessarily make it survive?

Therapist 14:25

So really, behaviors that yeah, behaviors that how did you phrase the behaviors that make it flourish and the other ones that don't make it survived like behaviors that don't make it survive? Well, blaming, criticizing demean like, how do you say diminishing or the value of what someone is saying or feeling? Yeah, this content despise this? Yeah, this criticism or defensiveness, it's also. Yeah. So these are kinds of behaviors that make it very difficult for relationship to survive. When people are there. Don't they?

Interviewer 15:22

Okay and the main thing that you were talking about when we talked about relationship quality is really, you know, like attending to your partners. Does how you conceptualize relationships often differ from how your clients conceptualize relationships? And how does this play out in therapy?

Therapist 15:54

They conceptualize relationships, I don't know if it comes so different how they conceptualize relationships. I think that's I think what, what, maybe what plays more is? And I don't know if that answer your questions, but maybe what plays more is the this framing problems as communication problems, and maybe not seeing the bigger picture of the need to feel safe in a relationship, that that maybe this plays a role when when in my work, like to try to make help clients understand this bigger picture that it's not possible to change the way they interact only seeing as a communication problem, but as but seeing that, if they don't feel safe, they won't, they won't be able to have a different different exchange. So I think that's how it plays. So it's a way of framing. But I don't know if Yeah, if I answered your question, if they..

Interviewer 17:22

It makes sense that they just see it as a communication problem, instead of some underlying need that they need to fill in the gaps for each other, I would say, yes, yes. Another question I have is that relationship is a complex system, right? And involves love involves sex identity, relationship maintenance, conflict responsiveness and all that drip therapy? How do you decide which one to focus on? Which one of this complex system to target?

Therapist 17:58

You mean, the topics, if they are if they bring sex, or this?

Interviewer 18:05

Yes, right. Like we have so many things involved in the relationship? How do you? How do you prioritize which one to focus on?

Therapist 18:16

I, I, for me, it doesn't matter so much the like the contents of ..like, if they bring sex or financial problems, I welcome all different topics as a way of understanding and bringing them to see this bigger picture of what they are not getting from their partner in that moment that matters to them. So I this I welcome all of them. But being explicit that I'm not focusing on, on on topics, because that's something that's only possible more towards the end of a therapy to solve problems, specific problems if they if they get there in therapy, but it's, I say that I focus on the emotional connection. So that's my focus. And yeah, and maybe they bring that as different topics, but in the end, that's where I am always focusing on their emotional connection with each other.

Interviewer 19:41

Interesting, okay. And when do you how do you determine when to end therapy?

Therapist 19:51

It's, I think, for in my experience, most of the times I think clients decide if that they, that what they did was, was helpful or and they they stop. But typically, it's when they, they, they are not only de-escalated, like in the sense that they are not having intense fights, but that they feel that they can do this. They, they are aware that they, they have to turn to their partner and listen if they have conflicts. And so it's it's basically this moment when they see that.

Interviewer 20:48

And usually how long does that take to get to that moment where not de-escalated, but they have the skills to do it.

Therapist 20:57

I find this a very difficult question. Very, very difficult, because I I personally, I know, some therapists may say this takes this time, but I I personally don't have that. Because it depends. It depends so much. And, of course, many clients don't get there, don't they, You know, they get divorced, they separate or they continue to live the way they will live? I don't know that there are so many things that happened. And so, I don't know, I don't know, for some clients, it can. A few sessions can feel very powerful. And they can make changes, but with others, it can take many sessions and and you don't see that change. So I find it very difficult to say that.

Interviewer 21:51

And usually how many times, for example, I'm just trying to get a sense of the practice of how often they come. Is it once a week, once every two weeks?

Therapist 22:03

Most of the times once every two weeks? Yeah, frequency is, in my practice has been difficult. They are always busy, difficult to find time. So ideally, it's once a week. But what happens is once every two weeks, or even a bit less, less frequent.

Interviewer 22:33

So some couples, take only a few sessions before they determined to end the therapy. Well, some, would you say? Would there be couples that take a year? More than a year?

Therapist 22:46

Yeah. All right. They are.

Interviewer 22:49

Okay. And as you know, of God's hands would have already told you we are currently developing a relationship quality scale that hopefully better represents the relationship as a complex system that will hopefully help therapist and treatment planning and evaluation. How interested would you be in the scale?

Therapist 23:09

Very

Interviewer 23:12

And like I because we are still developing it? Of course, we would want to get your view as well. What would you be looking for in a newly developed relationship scale like this?

Therapist 23:31

Well, I think it's, it's I think I would be looking to Yeah, understanding where they are in their relationship when they start therapy, and potentially as a way to also throughout therapy. Check where they are, again, you know, to kind of see the progress of the work. Yeah, so I think that that could be something that helps to see where we are in those aspects. And but yeah, but also, well, potentially as a way to guide to guide the work and see what are the things that maybe and I think that will be also new for for at least EFT but maybe that changes also the way we do therapy and we'd say okay, we have to focus on maybe on this whatever, whatever it is that the scale can kind of point to maybe a direction that can change the work the way we do the work. Yeah. And and also as a way to bring clarity for clients. So things that matter in a relationship, I think that's really important.

Interviewer 25:13

All right, all right, because of course, you want this scale to be as useful as possible for the therapists. So if, with what you were saying, you know, scale should help you understand where the clients are in their relationship as a sort of progress checks as well, during the relationship during the course of therapy, and potentially indicate direction of where the therapy, the direction of the therapy should go, as well as understandable or bring clients of clarity for the clients about their their issues as well. I wanted to ask as well, because when you would integrate this in your practice, or I'd say would, I'm trying to visualize how this would fit into your future clinical practice, if ever? Would there be time in your sessions to integrate it in your either in form assessment or evaluation of the clients during the beginning of the therapy?

Therapist 26:18

Yes, yes. It's yeah, I, I don't I don't understand exactly how it will work. But if it will be something that I will do as part of the assessment that I could do as part of the assessment, but yes, I think ...

Interviewer 26:40

Would you imagine this working better as an online thing as well. Or offline. As like an online,

Therapist 26:47

Like a self self. Yeah. Yeah, I could see that. I mean, I've seen recently was, and I know, it's not validated or anything, but it was, like just some tests from Gottman that he has, like an online tool for therapists that you they can go there. And it's they have many skills, which are not validated or but anyways, it's I thought it was interesting if Yeah, clients can do that way, I think. But, but yeah, if it's not, yeah, or something that it can be integrated in something that I do directly with them. But as I see many clients online, for me, it ends also sometimes to make sense that they do that. I don't know, online. No way.

[Portion Redacted]

Interviewer 29:04

All right. All right. Thank you so much. Of course, that's not everything is decided yet, but we're still trying to see, of course, how this would play out, logistically as well.

Therapist 29:17

Yes, yes.

Interviewer 29:18

All right. And I assume I know the answer to this. But I need to ask again. Would you be interested in collaborating in the development of this scale further collaborating aside from that support?

Therapist 29:31

Yeah. Yes

Interviewer 29:33

All right. Before we end, I just like to go over the things that we have discussed and maybe if you want to plug in some more if there's a thing that you haven't said, then you can do so in this review. We started with discussing your clinical practice, you primarily provide EFT especially to expats I usually one of them is [Nationality A], one of them is [Nationality B]. And a Well, here in [Country A], couples here in [Country A] usually are your clients is the usual presenting issue to you or problems in communication, they experience stress during when they interact. You don't do a formal assessment, however you're Well, your assessment procedure basically is to ask your, your clients directly to understand where they're coming from examples of interaction, how they behave, what they feel about their behaviors, their attributions of their behaviors, and also their attachment history, both to their romantic partners, and to other attachment figures, like parents, etc. You've also done you've also used relationship scales in the past by Sue Johnson, only a few times, you didn't find that helpful, mostly because it was a yes or no as well. And it was more as a starting point for for discussion with your clients rather than something informative in itself. And when we talked about relationship quality for you, really, the main thing is that the your hurt should matter to your partner, basically, for you that is that that is what your relationship quality is. And other important things in our relationship include being seen, being valued being loved, and what will destroy a relationship of crisis blaming, criticizing diminishing your partner, and so on. When, when talking about maybe possible differences in how you conceptualize relationships with their client, you said that there wasn't too much of a difference. It was more of a they see it as a communication problem where you see it as a symptom of a deeper underlying need, for example, need to feel safe, etc. And when you when talking about relationship as a complex system, for you will no matter what topic, finance, etc, it's really the emotional connection that you're focusing on, I would believe the process of the emotional interaction that you're focusing on more than the topic itself. And when you determine the end of therapy, usually the clients decide when they've already de- escalated and basically have the capacity for this healthy, new emotional interactions that EFT provides. You are interested in this develop this newly developed relationship scale as a sort of baseline of where they are as a sort of check during therapy, as clarity for the clients as well and possible pointing of direction. You think that scale? Well, you think it's expensive, but if it helps the work of the client, then it should be worth it as an investment. Anything you want to add in the thing that we've talked about?

Therapist 33:09

Yeah, I was thinking I was just thinking about two things, as you said everything. I think, at the end, when you said about the scale, what I thought was also, as I said, I think it's for for me as as a therapist, but that follows, for example, a model EFT that I tried to work according to the model, I think it's really important when something is kind of integrated with the work. So I was just thinking, as you said about the scale that this was something that just just kind of that I that I thought like how the scale would kind of be integrated into the the approach. Like if if you start seeing trainers that that do EFT that supervise EFT showing how they use it and how this can be integrated to the work we do that something because we are constantly learning of course, at least. Ideally, we therapists are always doing supervision and having other people that are more experienced or just peer colleagues, senior work I think if it's something that it's becomes also integrated in the model that that people say okay, we are using these and how we are doing and how we can use this to better do the work that we do. I think that's important because we use a model as a map. And and we have a way that we learn to do things that if this is not integrated, even though it can be something very interesting, it can be difficult for us to, at least I'm speaking for me to say how but what do I do? How do I do EFT with these? That it's, it's so interesting, but how do I, how do I use that? How this will help me so? So yeah, I was just thinking about that about how to when, when it's there, how to make it accessible and kind of talk, you know, I don't know, to people that are trainers in the model to understand how this could be integrated into, into trainings into because then it's Yeah. Then if it's integrated, everybody's doing the same thing. You know, it's, we do we know what we are doing. I don't know if it makes sense. But it's. And the the other thing that I thought, yeah, it was just about what relationship quality means. And yeah, I think it's such a big question. And I think I'm sure I missed, like, what what does it what does it mean, in a full sense, but I yeah, I talked about the hurt the hurt matters. That as a sign, but yeah, I think it's is relationship quality, I think it's measured by how you feel safe, how you, like, if you feel safe in a relationship, if you I think that's the quality because that's, yeah, that's how you can basically thrive and have the potential to, to explore to expand to grow to so if you don't have that same, like relationship quality for me, it's really it's related to feeling safe. Not only in a romantic relationship, but in any in a therapeutic relationship. It's also about feeling safe feeling that you...

Interviewer 37:07

When do you know, if you're safe in a relationship?

Therapist 37:12

Yeah, with you know, you're safe. When you your matter, you I feel it's related to that when you can show your spots you can you can show things that maybe you things that matter to you things that are important to you, but also things that maybe are difficult for you to show to others. And this is this is you, you have this, the feeling safe is that you feel that you can do this, even though it's you that there is not a sense of threat, if you show something I think the feeling safe is related to that if I if I feel hurt or if I feel vulnerable, or if there is something difficult, I can show and this will this will be it's not about solving and having, but it's someone will see me and will be there.

Interviewer 38:31

That's really interesting. Okay, okay. Okay. Okay. Great. Anything else that you wanted to add?

Therapist 38:42

Oh, I'm just curious about how, I don't know. I'm just very curious how because I feel when I'm talking it's yeah, it's a bit. Not so objective. So I'm very curious how you will turn all the all these are what other therapies and other people say and everything you know about relationships into? Yeah, into objective...

Interviewer 39:11

Yes, definitely. But before I respond to that, before we end as well, because I wanted to talk to you about that after the recording. I wanted to talk to you about how we will handle the data. Of course with we want. We want our project to be a research project to be as transparent as possible and as shareable as possible. So I wanted to ask you which of the data sharing options you're most comfortable with? One option is that transcript portions can be shared, none of which can be identified to you. Another option, if you were bolder, is that the whole transcript can be shared and removed of sensitive and overly identifying details. So it will be anonymized. Although in this option, there is a chance that someone who knows you well professionally can piece together the details. was like, Oh, they had the practice in the [Country C], [Country B] and might be able to narrow down among colleagues about who they believe it is. Although, again, no sensitive information will be shared, although when I'm reviewing our discussion, I don't believe there's any anything here that is overly sensitive or can be

Therapist 40:21

No.

Interviewer 40:23

Yes. And

Therapist 40:24

it's okay for me if someone Yeah, the later the later options like if someone can identify me by

Interviewer 40:36

Perfect, I'll be sending you a copy and if you want anything redacted I will redact it. Alright, I will end the recording and I will tell you a bit of my thoughts about where we are currently..